Where the slides differ from current guidance

ICC-1, Year 3, 2026-2027. 19 points to discuss with your doctor or tutor.

How to use this. In the quizzes, the correct answer always follows the slides, because the exam is written from them. For each point below, the slide and current guidance say different things. Take this list to your tutor and ask which answer the exam expects.

- **Where:** the lecture file name and the slide page number, so you can open the exact slide.
- **Question:** the quiz question ID that tests this point (shown at the bottom of each question in the quiz and in the CSV).

Cardiology

1. Clopidogrel loading dose in ACS and STEMI

Slide says	Give aspirin 300 mg plus clopidogrel 300 mg as the loading dose.
Current guidance	Many current guidelines (ESC) use a 600 mg clopidogrel load before primary PCI.
Where	ICC-1 [070,086] ECG Workshop with answers; ICC-1 [055] Cardiopresentations, Workshop p3-5 and p48; Cardiopresentations p3
Question	ICC1-CAR-016, ICC1-CAR-185

Ask: Which loading dose should I write in the exam: 300 mg or 600 mg?

2. When to repeat troponin

Slide says	Repeat troponin 10-12 hours after the start of the chest pain.
Current guidance	Current high-sensitivity troponin pathways repeat the test much sooner, for example at 0 and 1-3 hours.
Where	ICC-1 [028,064] ACS LGW final tutor notes, p3
Question	ICC1-CAR-124

Ask: Does the exam expect the 10-12 hour answer?

3. Rate control of AF with a weak heart (LVEF under 40%)

Slide says	Use beta-blockers or digoxin; avoid verapamil and diltiazem.
Current guidance	Textbooks agree, but warn to use IV beta-blockers carefully in acutely decompensated heart failure.
Where	ICC-1 [078] / [079] Tachyarrhythmias-Bradyarrhythmias, p13-14
Question	ICC1-CAR-031

Ask: Small point: is digoxin preferred while the patient is acutely unwell?

Chest

4. TB meningitis drug regimen

Slide says	Replace ethambutol with streptomycin; treat for 9-12 months.
Current guidance	Current WHO guidance still commonly keeps ethambutol in the regimen for TB meningitis.
Where	ICC-1 [114] Extrapulmonary Tuberculosis (TB), p21
Question	ICC1-CHE-055

Ask: Is the streptomycin swap still the expected answer?

5. Definition of massive haemoptysis

Slide says	More than 150 ml coughed up in one attack.
Current guidance	Textbook definitions vary widely, often 100-600 ml in 24 hours.
Where	ICC-1 [108] Respiratory Cardinal Symptoms, p46
Question	ICC1-CHE-181

Ask: Which figure should I quote?

6. Chest drain size for empyema

Slide says	A large-bore (20 French) chest tube is a must.
Current guidance	Some current guidelines (BTS) accept smaller-bore tubes for empyema.
Where	ICC-1 [117] Pleural Diseases LGW, p24 and p28
Question	ICC1-CHE-211

Ask: Is 'large-bore tube' the answer to give?

7. Anticoagulation for pulmonary embolism

Slide says	Start subcutaneous LMWH plus warfarin, target INR 2-3.
Current guidance	Current guidelines usually prefer a direct oral anticoagulant (such as apixaban or rivaroxaban) over warfarin.
Where	ICC-1 [098] Chest Cases Review, p17
Question	ICC1-CHE-224

Ask: Would a DOAC answer be marked correct?

Vertical

8. Radiology: Imaging the post-phlebitic (post-thrombotic) leg

Slide says	CT venography is the modality of choice for the post-phlebitic limb; Doppler for acute DVT.
Current guidance	Most standard references use duplex ultrasound first for the post-thrombotic limb as well.
Where	ICC1 [VM 024] Venous Imaging, p37 and p48
Question	ICC1-VER-130

Ask: Which test does the department expect here?

9. Mental health: Activated charcoal in paracetamol overdose

Slide says	Give activated charcoal if a serious overdose presents within 2 hours.
Current guidance	Many current references (UK TOXBASE, NICE) use 1 hour.
Where	ICC3 [VM 065] Self harm Suicide, p20
Question	ICC1-VER-241

Ask: Should I write 1 hour or 2 hours?

10. Mental health: When suicide risk is highest after self-harm

Slide says	Highest within 6 months of the self-harm episode.
Current guidance	Many references describe the highest risk in the first year (especially the first weeks).
Where	ICC3 [VM 065] Self harm Suicide, p23 and p33
Question	ICC1-VER-246

Ask: Is '6 months' the answer the exam wants?

11. Mental health: Which law applies (disturbed behaviour lecture)

Slide says	Uses UK law: Mental Health Act Sections 5(2), 5(4), 3 and the Mental Capacity Act.
Current guidance	In Egypt the Egyptian Mental Health Act (2009) applies, taught in a separate lecture [VM 084].
Where	ICC1 [VM 031] Management of disturbed behaviour, p16 and p23-24
Question	ICC1-VER-166 and other X15 questions

Ask: Will exam questions use UK sections, Egyptian articles, or both?

12. Mental health: Restarting an antipsychotic after neuroleptic malignant syndrome

Slide says	Wait at least 5 days before cautiously restarting, using a structurally unrelated drug.
Current guidance	Many references advise waiting about 2 weeks after NMS resolves.
Where	ICC1 [VM 031] Management of disturbed behaviour, p30
Question	ICC1-VER-158

Ask: Is 5 days the expected answer?

13. Mental health: Depression, anxiety and OCD case notes

Slide says	The student notes give case questions but no model answers.
Current guidance	My answers follow standard NICE teaching (CBT with exposure and response prevention for OCD, sertraline after a heart event, no long-term benzodiazepines).
Where	ICC1 [VM 042] Depression Anxiety and Obsession student notes, whole file
Question	ICC1-VER-167 to ICC1-VER-176

Ask: Can the tutor share the model answers to check against?

14. Use of medicine: Reversing warfarin when INR is 8 (answer file error)

Slide says	The case answers say 'Stop warfarin, protamine zinc, FFP'.
Current guidance	Protamine reverses heparin, not warfarin. Warfarin is reversed with vitamin K, and prothrombin complex concentrate (or FFP) if bleeding. The lecture itself (p14-15) is correct. I did not use the wrong answer.
Where	ICC1 [VM 034] Anticoagulant Therapy QA case review answers, p2
Question	none (not tested)

Ask: Please confirm the answer file has a mistake.

15. Use of medicine: Antidotes for the newer oral anticoagulants (NOACs)

Slide says	One lecture says NOACs have 'no specific antidote'. Another lecture names idarucizumab (dabigatran) and andexanet alfa (apixaban, rivaroxaban).
Current guidance	Both antidotes exist and are licensed. The two lectures disagree with each other.
Where	ICC1 [VM 034] Antiplatelets and Anticoagulant Therapy; ICC1 [VM 110] Clinical Therapeutics Cardiology, [VM 034] p36; [VM 110] p16
Question	not tested as a 'no antidote' question

Ask: If asked, should I name the antidotes?

16. Pathology vs Vascular surgery: Size for elective repair of an abdominal aortic aneurysm

Slide says	Pathology lecture: operate if large (over 5 cm) or growing. Vascular lecture: repair at 5.5 cm.
Current guidance	Current guidelines use 5.5 cm in men (5.0 cm is sometimes used in women), or fast growth, or symptoms.
Where	Final [037a] ATH and aneurysms; ICC-1 [085] Aneurysms, [037a] p63; [085] p39
Question	high yield sheets only

Ask: Which cut-off should I give: 5 or 5.5 cm?

Hematology

17. Type of haemolysis in cold autoimmune haemolytic anaemia

Slide says	Mostly intravascular, complement-mediated haemolysis.
Current guidance	Most textbooks describe it as mainly extravascular (C3b-coated cells removed in the liver); intravascular lysis only when severe.
Where	ICC-1 [077] Haemolytic anaemia, p11
Question	ICC1-HEM-081

Ask: Which description does the exam expect?

18. Cardiovascular risk calculator

Slide says	NICE recommends QRISK2.
Current guidance	Current NICE guidance uses QRISK3 (QRISK2 was replaced).
Where	ICC-1 [027] Clinical chemistry tutorial Q and answers, p25-29
Question	ICC1-HEM-125

Ask: Is QRISK2 still the expected answer?

19. Warfarin skin necrosis: which factors fall later than protein C

Slide says	Protein C falls before factors II, VII, IX and X.
Current guidance	Factor VII has a half-life as short as protein C, so the accurate comparison is with II, IX and X.
Where	ICC-1 [008]b Thrombophilia inherited and acquired part 1, p37
Question	ICC1-HEM-002

Ask: Minor point, for understanding only.